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PURE PATH LAB
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Dr. Abha Mishra
M.B.B.S (M.D. PATH)
Reg. No-MP-8779
PURE PATH LAB
Palce of Accuracy & Your Satisfaction

Mrs. Ayesha
Age / Sex : 30 YRS / Female
Reg. no. : 6961

Registered on : -
Collected on : -
Received on : -
Reported on : -

HAEMATOLOGY
COMPLETE BLOOD COUNT (CBC)

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN	**L**	**8.2**	**g/dl**	**12 - 15**
TOTAL LEUKOCYTE COUNT	**H**	**16000**	**cumm**	**4,800 - 10,800**
NEUTROPHILS		57	%	40 - 80
LYMPHOCYTE	**L**	**13**	**%**	**20 - 40**
EOSINOPHILS	**H**	**28**	**%**	**1-6**
MONOCYTES		02	%	2 - 10
BASOPHILS		00	%	<2
PLATELET COUNT	**L**	**0.62**	**lakhs/cumm**	**1.5 - 4.1**
TOTAL RBC COUNT	**L**	**2.09**	**million/cumm**	**3.9 - 4.8**
HEMATOCRIT VALUE, HCT	**L**	**21.6**	**%**	**36 - 46**
MEAN CORPUSCULAR VOLUME, MCV	**H**	**103.3**	**fL**	**83 - 101**
MEAN CELL HAEMOGLOBIN, MCH	**H**	**39.2**	**Pg**	**27 - 32**
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**38.0**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV		10.6	fL	6.5 - 12
R.D.W. - SD	**H**	**67.9**	**fL**	**39 - 46**
R.D.W. - CV	**H**	**19.2**	**%**	**11.6 - 14**
P-LCR		35.6	19.7	- 42.4
P.D.W.	**H**	**17.5**	**fL**	**9.6 - 15.2**

Possible causes of abnormal parameters

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

BIOCHEMISTRY
BIOCHEMISTRY

TEST	VALUE	UNIT	REFERENCE
RANDOM BLOOD SUGAR	89.67	mg/dl	70 - 140

BIOCHEMISTRY
KIDNEY FUNCTION TEST (KFT)

TEST		VALUE	UNIT	REFERENCE
SERUM UREA	**H**	**112.6**	**mg/dl**	**13 - 40**
SERUM CREATININE	**H**	**12.17**	**mg/dl**	**0.55 - 1.13**
SERUM PHOSPHORUS	**H**	**10.58**	**mg/dl**	**2.5 - 4.5**
SERUM URIC ACID	**H**	**9.30**	**mg/dl**	**2.6 - 6**
SERUM SODIUM	**L**	**134.5**	**mmol/L**	**136 - 146**
SERUM POTASSIUM		4.68	mmol/L	3.5 - 5.1
ICALCIUM	**L**	**1.10**	**mmol/l**	**1.13 - 1.33**

Creatinine is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

Causes of Increased Serum Creatinine Level	Causes of Decreased Serum Creatinine Level
1. Pre-renal, renal, and post-renal azotemia	1. Pregnancy
2. Large amount of dietary meat	2. Increasing age (reduction in muscle mass)
3. Active acromegaly and gigantism	

~~~ End of report ~~~
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